

GRAIN DIRECTION



SAME SIZE ARTWORK
LEAFLET: 225 mm x 270 mm
FOLDING SIZE: 38 mm x 135 mm



135 mm

SPACE FOR PHARMACODE

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For the use only of a Registered Medical Practitioner or a Hospital or a Laboratory

Ibicar S

CFC FREE

Salbutamol 100mcg/actuation and
Beclometasone 50mcg/
actuation Pressurised Inhalation)

COMPOSITION:

Each actuation delivers:

Salbutamol Sulphate BP equivalent
to Salbutamol

100 mcg

Beclometasone Dipropionate
Anhydrous Ph. Eur

50 mcg

Ethanol BP

3.5% w/w (5.23% v/v) (2.52 mg)

DOSAGE FORM

Inhalation aerosol

DESCRIPTION

White to off white colour suspension filled in aluminium aerosol can with silver ferrule valve provided with actuator suitable for oral inhalation when actuated a metered spray is produced.

PHARMACOLOGY

Pharmacodynamics

Salbutamol:

Salbutamol is a selective beta-2 adrenoceptor agonist. At therapeutic doses it act on the beta-2 adrenoceptors of bronchial muscle.

Beclometasone dipropionate:

Beclomethasone dipropionate given by inhalation has a potent glucocorticoid anti-inflammatory action within the lungs.

Pharmacokinetics

There is no evidence that the administration of salbutamol and beclomethasone dipropionate together by the inhaled route in any way affects the pharmacokinetics of either component. For pharmacokinetic purposes therefore each component can be considered separately.

Salbutamol:

Salbutamol administered intravenously has a half-life of 4 to 6 hours and is cleared partly renally and partly by metabolism to the inactive 4'-O-sulphate (phenolic sulphate) which is also excreted primarily in the urine. The faeces are a minor route of excretion. The majority of a dose of salbutamol given intravenously, orally or by inhalation is excreted within 72 hours. Salbutamol is bound to plasma proteins to the extent of 10%.

After administration by the inhaled route, between 10 and 20% of the dose reaches the lower airways. The remainder is retained in the delivery system or deposited in the oropharynx from where it is swallowed. The fraction deposited in the airways is absorbed into the pulmonary tissues and circulation but it is not metabolized by the lung. On reaching the systemic circulation it becomes accessible to hepatic metabolism and is excreted, primarily in the urine, as unchanged drug and as phenolic sulphate.

The swallowed portion of an inhaled dose is absorbed from the gastrointestinal tract and undergoes considerable first pass metabolism to the phenolic sulphate. Both unchanged drug and conjugate are excreted primarily in the urine.

Beclometasone dipropionate:

Beclomethasone 17, 21-dipropionate (BDP) administered intravenously is cleared rapidly with a half-life of approximately 30 minutes. Beclomethasone 17-monopropionate (BMP) appears rapidly in the plasma after intravenous BDP and is itself cleared with a half-life again of about 30 minutes. BDP is bound to plasma proteins to the extent of 87%. Up to 14% of an intravenous dose of BDP is excreted in the urine in 96 hours, mainly as polar metabolites, a proportion of which are conjugated. Upto 64% of the dose is

excreted in faeces in his time, again primarily as free and conjugated metabolites.

After an inhaled dose, the majority of the dose will be deposited in the oropharynx or on the delivery device, with only a fraction reaching the lungs and available for absorption from this site. That portion deposited in the mouth or upper airways will ultimately be swallowed.

The lung tissue rapidly hydrolyses BDP to BMP which in turn is hydrolyzed more slowly to beclomethasone. The liver also metabolizes BDP to BMP and further converts it to polar metabolites.

INDICATIONS

Ibicar S is suitable for those patients who require regular doses of both salbutamol and beclomethasone for treatment of reversible airways obstruction. Ibicar S is not intended for use as first line treatment but is for use once the need for inhaled corticosteroid therapy has been established.

DOSAGE AND ADMINISTRATION

Salbutamol and Beclometasone Pressurised Inhalation may be used with a Spacer device by patients who find it difficult to synchronize aerosol actuation with inspiration of breath.

Adults - Two inhalations, three or four times daily, titrated to the lowest effective dose. The maximum dose is two inhalations four times a day.

Children - One or two inhalations, two, three or four times daily. The maximum dose is two inhalations four times a day.

Elderly - There is no need to adjust the dose in elderly patients.

CONTRAINDICATIONS

Hypersensitivity to any of the components of the formulation. Ibicar S is not appropriate for managing premature labour.

Pregnancy- Category C:

Administration of this combination of drugs during pregnancy should only be considered if the expected benefit to the mother is greater than any possible risk to the foetus.

Lactation-

This combination should be used only if the expected benefit to the mother is likely to outweigh any potential risk to the neonate.

ROUTE OF ADMINISTRATION

Oral Inhalation

SIDE EFFECTS

Systemic effects of inhaled corticosteroids may occur, particularly at high doses prescribed for prolonged periods. These may include adrenal suppression, growth retardation in children and adolescents, increase in bone mineral density, cataract and glaucoma. As with other inhalation therapy, paradoxical bronchospasm may occur with an immediate increase in wheezing after dosing. This should be treated immediately with a fast-acting inhaled bronchodilator. Ibicar S should be discontinued immediately, the patient assessed, and if necessary, alternative therapy instituted.

Hoarseness, mouth and throat irritation may occur, it may be helpful to rinse the mouth with water immediately after inhalation.

Hypersensitivity reactions, including angioedema, bronchospasm, hypotension and collapse have been reported very rarely. Rashes, urticaria, pruritus and erythema and oedema of the eyes, face, lips and throat, have also been reported.

Salbutamol:

Salbutamol may cause a fine tremor of skeletal muscle; usually the hands are most obviously affected. This effect is dose related and is common to all Beta-adrenergic stimulants. Headaches have occasionally been reported. Tachycardia, with or without peripheral vasodilatation, may rarely occur. In common with other Beta2 agonists, cardiac arrhythmias (including atrial fibrillation, supraventricular tachycardia and extrasystoles) have

been reported in association with use of salbutamol, usually in susceptible patients.

There have been very rare reports of muscle cramps. Potentially serious hypokalaemia may result from Beta2 agonist therapy.

As with other Beta2 agonists, hyperactivity in children has been reported rarely.

Beclomethasone dipropionate:

Candidiasis of the mouth and throat (thrush) occurs in some patients inhaling beclomethasone dipropionate, the incidence increases with doses greater than 400 mcg beclomethasone dipropionate per day. Patients with high blood levels of *Candida precipitins*, indicating a previous infection, are most likely to develop this complication. Such patients may find it helpful to rinse their mouth with water after using the inhaler. The condition usually responds to topical anti-fungal therapy without discontinuing treatment with Ibicar S.

WARNINGS AND PRECAUTIONS

Patients should be instructed in the proper use of the inhaler to ensure that the drug reaches the target area within the lungs. They should also be made aware that Ibicar S must be used regularly, every day, for optimum benefit. Patients should be regularly reassessed so that their continuing need for corticosteroid therapy can also be reviewed.

Ibicar S is not for use in acute attacks but for routine long-term management, so patients will require a fast and a short-acting inhaled bronchodilator to relieve acute symptoms. However, should the effect of the additional inhaled bronchodilator or the relief provided by the Ibicar S last for less than three hours, patients should be advised that this may indicate that their condition is worsening and to seek medical advice, in case treatment with inhaled corticosteroid needs to be increased, or treatment with systemic corticosteroid needs to be started or increased. As there may be adverse effects associated with excessive dosing, the dosage of frequency of administration should only be increased on medical advice.

Severe asthma requires regular medical assessment, including lung function testing, as patients are at risk of severe attacks and even death. Increasing use of short-acting inhaled Beta2 agonists to relieve symptoms indicates deterioration of asthma control. If patients find that short acting relief bronchodilator treatment becomes less effective or they need more inhalations than usual, medical attention must be sought.

For those patients who are steroid-dependent it is advisable to commence therapy with beclomethasone dipropionate alone.

Patients who have been weaned in the previous few months from long term systemic corticosteroids need special consideration until the hypothalamic pituitary adrenal (HPA) system has recovered sufficiently to enable the patient to cope with emergencies such as trauma, surgery or infections. Such patients should carry a steroid warning card indicating that they may need supplementary systemic steroid during periods of stress, until their adrenocortical function has become normal. These patients should also be given a supply of oral steroid to use in emergency when their airways obstruction worsens.

Systemic effects of inhaled corticosteroids may occur, particularly at high doses prescribed for prolonged periods. These effects are much less likely to occur than with oral corticosteroids. Possible systemic effects include adrenal suppression, growth retardation in children and adolescents, decreased in bone mineral density, cataract and glaucoma. It is important therefore that the dose of inhaled corticosteroid is titrated to the lowest dose at which effective control of asthma is maintained.

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25 mm

ICONGRAPHICS CODE:

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Artwork Code
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It is recommended that the height of children receiving prolonged treatment with inhaled corticosteroids is regularly monitored. If growth is slowed, therapy should be reviewed with the aim of reducing the dose of inhaled corticosteroid, if possible, to the lowest dose at which effective control of asthma is maintained. In addition, consideration should be given to referring the patient to a pediatric respiratory specialist.

Prolonged treatment with high doses of inhaled corticosteroids, particularly higher than the recommended doses, may result in clinically significant adrenal suppression. Additional systemic corticosteroid cover should be considered during periods of stress or elective surgery.

As with all inhaled corticosteroids, special care is necessary in patients with active or quiescent pulmonary tuberculosis.

Salbutamol should be administered with caution to patients suffering from thyrotoxicosis. Salbutamol and non-selective Beta blocking drugs, such as propranolol, should not normally be prescribed together.

Potentially serious hypokalaemia may result from Beta2 agonist therapy, mainly from parenteral and nebulizer administration. Particular caution is advised in acute severe asthma, as this effect may be potentiated by hypoxia and by concomitant treatment with xanthine derivatives, steroids and diuresis. Serum potassium levels should be monitored in such situations. Treatment should not be stopped abruptly.

DRUG INTERACTIONS

Beta-adrenergic blocking drugs inhibit the bronchodilator action of salbutamol and other sympathomimetic bronchodilators. Such drugs should not be used in asthmatic patients as they may increase airway resistance. Salbutamol is not contra-indicated in patients who are under treatment with monoamine oxidase inhibitors. There is no known drug interaction with beclomethasone.

OVERDOSAGE

Salbutamol

The preferred anti-dote for overdose with salbutamol is cardioselective beta blocker which should be used with caution in patients with patients with bronchospasm. Serum potassium should be monitored as hypokalemia may occur with overdose of salbutamol.

Beclomethasone dipropionate

Acute- If higher than recommended dosage is continued over prolonged periods, some degree of adrenal suppression may result. This does not require immediate action. In these patients treatment should continued at a dose sufficient to control asthma as adrenal function recovers in a few days and can be verified by measuring plasma cortisol.

Chronic- Inhalation of Beclomethasone in excess of 1500 mcg over prolonged period may lead to adrenal suppression. Monitoring of adrenal reserve may be necessary. In cases of beclomethasone dipropionate overdose, therapy may still be continued at a suitable dosage for symptom control.

STORAGE

Store below 30°C. Do not freeze.

PACKAGING INFORMATION

Canister containing 200 metered doses

SHELF LIFE

24 months

Date of Revision:

1 November 2019

Mal No.: MAL14115007AZ

INSTRUCTIONS FOR PATIENT FOR USING THE INHALER WITHOUT SPACER

Important: Follow instructions carefully.

Shake the inhaler well immediately before each use.

Testing your inhaler:

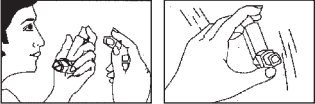
If you are using the inhaler for the first time or if the inhaler has not been used for a minimum of two days, "test spray" the inhaler. Spray the inhaler 4 times into the air.

(see pic no. 1)



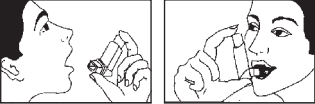
1

Remove the cap from the mouthpiece; the strap on the cap will stay attached to the actuator. If the strap is removed from the actuator, the mouthpiece should be inspected for the presence of foreign objects before each use. Make sure the canister is fully and firmly inserted into the actuator. (see pic no. 2)



2

Hold the inhaler upright with your thumb on the base. Place either one or two fingers on the top of the canister. Breathe out fully through your mouth expelling as much air from your lungs as possible. Thereafter, place the mouthpiece of the inhaler in your mouth between your teeth. (see pic no. 3)



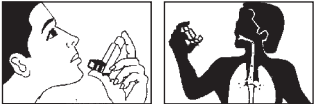
3

Close your lips around it (do not bite it) tilt your head slightly backwards. Start breathing in slowly through your mouth. As you breathe in steadily and deeply, press down the canister to release one dose. (see pic no. 4)



4

Remove the inhaler from your mouth and hold breath for 10 seconds or for as long as it is comfortable. Breathe out slowly. (see pic no. 5)



5

Note:

Rinse your mouth or gargle with water after inhaling each dose. This is likely to reduce the soreness that may be caused by the drug. If another dose is required wait for at least one minute and repeat steps 2 through 5 for each inhalation prescribed by your Physician. After use, replace the mouthpiece cover. (see pic no.6)



6

Practice in front of the mirror for the first few times. If you see 'mist' coming from top of the inhaler or sides of the mouth, this indicates failure of technique. Start again from Step 2. (see pic no. 7)



7

For Children:

Children should use the inhaler under adult supervision, as instructed by the Physician. (see pic no.8)



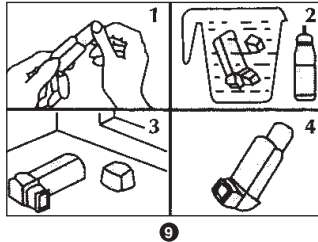
8

Cleansing:

Clean the inhaler at least once a day.

1. Gently pull the metal canister out of the plastic body of the inhaler. Remove the mouthpiece cover.
2. Rinse the plastic body and the mouthpiece cover in warm / running water.
3. Dry thoroughly. Avoid excess heat.
4. Replace the canister and the mouthpiece cover correctly.
5. Store the spacer by hygienically.

Discard the canister after using the labeled number of inhalation.



9

Product Registration Holder

Glenmark Pharmaceuticals (Malaysia)
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